

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Nalmefene 18mg tablets (Selincro) for the treatment of Alcohol Reduction and Dependence Management

Summary of NICE / NICE CKS Guidelines for Alcohol Reduction and Dependence Management

Overview

- Alcohol dependence and harmful drinking are common health conditions
- NICE CG115 provides comprehensive guidance on management
- Treatment should address both physical and psychological dependence

Assessment

- Use AUDIT (Alcohol Use Disorders Identification Test) score to assess severity
- Quantify units of alcohol consumed per week
- Assess degree of physical and psychological dependence
- Perform baseline liver function tests
- Assess patient motivation to reduce or abstain from alcohol

Management

- Brief interventions are first-line for mild-moderate dependence
- Specialist referral indicated for severe dependence requiring medically-assisted withdrawal
- Pharmacological support (nalmefene) combined with psychosocial interventions for moderate dependence
- Psychological support including CBT, family therapy, and peer support as appropriate

Red flags requiring urgent specialist referral

- Severe dependence requiring medically-assisted withdrawal
- Risk of delirium tremens or seizures
- Signs of Wernicke encephalopathy (confusion, ataxia, ophthalmoplegia)
- Acute liver failure or severe cirrhosis
- Severe comorbid psychiatric disorder

Patient Group Direction

for the supply/administration of

Nalmefene 18mg tablets (Selincro)

for the treatment of

Alcohol Reduction and Dependence Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Reduction of alcohol consumption in adult patients with alcohol dependence who have a high drinking risk level (DRL >60 g/day for men, >40 g/day for women per WHO criteria) without physical withdrawal symptoms, and who do not require immediate detoxification. Used together with continuous psychosocial support focused on adherence and reducing alcohol consumption.
Inclusion criteria	Age 18 years or over High drinking risk level (DRL >60 g/day men / >40 g/day women) confirmed on AUDIT and unit-counting. Patient should still be drinking (without acute withdrawal) at consultation. Demonstrated motivation to reduce alcohol consumption or abstain Not currently in acute alcohol withdrawal A pre-treatment abstinence period is NOT required for nalmefene (unlike nalmefene). Treatment can be initiated while the patient is still drinking, provided no acute withdrawal symptoms are present. Acceptable liver function (ALT and AST less than 3 times upper limit of normal) Informed consent given and documented
Exclusion criteria	Currently taking ANY opioid medication (prescription analgesics, opioid replacement, codeine-containing OTC). Nalmefene is an opioid

	receptor modulator and will precipitate severe acute withdrawal in opioid-dependent patients. Patient must cease opioids before treatment. Acute hepatitis or acute liver failure Liver function impairment with ALT or AST >5 times upper limit of normal Severe renal impairment (eGFR <30 mL/min) Pregnancy or breastfeeding Known hypersensitivity to nalmefene or to any of the excipients (lactose monohydrate, magnesium stearate). Patients with acute alcohol withdrawal symptoms, or who require immediate medically-assisted detoxification.
Cautions	Hepatic impairment: nalmefene is metabolised by the liver. Avoid in severe hepatic impairment (Child-Pugh C). Monitor LFTs at baseline and at month 3 in mild-moderate impairment. Renal impairment: dose reduction may be needed for eGFR <50 mL/min Concurrent opioid medications: nalmefene will block opioid analgesia and may precipitate withdrawal. Patient should temporarily discontinue nalmefene at least 1 week before any planned opioid analgesia (e.g. surgery). Patient should carry a medical alert card / wallet note stating they take nalmefene (which blocks opioid analgesia in emergencies, e.g. RTC, post-op). Potential for depression or mood changes; counsel patient to report mood disturbances Nalmefene is taken as-needed (PRN), so no taper is required when stopping. Refer rebound drinking back to GP.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Nalmefene 18mg tablets (Selincro)
Legal category	POM
Storage	Store below 25°C. Protect from light and moisture.
Route/method of administration	Oral. Swallow tablet whole with water. Take 1–2 hours BEFORE the anticipated start of drinking (or as soon as possible after the start of drinking, if the patient forgot the pre-drinking dose).

Dose and frequency	ONE 18 mg tablet on each day the patient anticipates a risk of drinking alcohol. Take 1–2 hours BEFORE the anticipated start of drinking. Maximum dose: ONE tablet in any 24 hours. The medicine is intended for as-needed (PRN) use and should NOT be taken every day if the patient does not anticipate drinking that day.
Quantity to be administered and/or supplied	Up to 28 tablets per supply. Quantity should reflect the patient's anticipated drinking-day frequency (e.g. 14 tablets for 3–4 drinking days/week).
Maximum or minimum treatment period	Review at 1 month and 3 months in the first 6 months. Continue if the patient demonstrates a meaningful reduction in alcohol consumption and tolerates treatment well. Treatment beyond 1 year requires GP / specialist review. No tapering required when stopping — the medicine is taken PRN.
Adverse effects	Very common (>10%): Nausea, headache, insomnia Common (1-10%): Abdominal pain, vomiting, anxiety, nervousness, joint pain, muscle pain, dizziness, decreased appetite, fatigue Uncommon (0.1-1%): Depression, irritability, chest pain, palpitations Rare (<0.1%): Hepatotoxicity (dose-dependent, reversible with dose reduction or discontinuation)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with nalmefene. Ensure patient understands it is an adjunct to psychological support, not a sole treatment.
Follow-up advice to be given to patient or carer	Do not take opioid medications while on nalmefene; contact GP or pharmacist before taking any painkillers. Carry your medical alert card at all times to inform healthcare professionals that you take nalmefene. Report any mood changes, depression, or suicidal thoughts immediately. Attend all follow-up appointments for liver function monitoring (blood tests at 2 weeks then monthly). Maintain engagement with psychological support (counselling, CBT, or group support). Do not suddenly stop nalmefene without consulting your healthcare provider. Inform all healthcare providers and dentists that you take nalmefene. Seek immediate medical attention if you experience severe abdominal pain, yellowing of skin or eyes, or dark urine.

Key references

• NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2 • NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources • NICE CKS Guidance: Alcohol use disorders (NICE CG115) • British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines
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Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025
